Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study to Learn About the Study Medicine PF-07934040 When Given Alone or With Other Anti-cancer Therapies in People With Advanced Solid Tumors That Have a Genetic Mutation **Official Regulatory Title:** A PHASE 1 OPEN-LABEL STUDY OF PF-07934040 AS A SINGLE-AGENT AND IN COMBINATION WITH OTHER TARGETED AGENTS IN PARTICIPANTS WITH ADVANCED SOLID TUMORS HARBORING MUTATIONS IN THE KRAS GENE **Short Title/Acronym:** Phase 1 Study of PF-07934040 in Subjects with Adv Solid Tumors with KRAS Mutation **Protocol Number (Primary ID):** PRO00116192 / Internal Pfizer Protocol **NCT Number (Registry Number):** NCT06447662 **Sponsor Name / Lead Organization:** Pfizer **Investigational Product:** PF-07934040 (panKRAS inhibitor) **Phase of Development:** Phase I **Medical Monitor:** Pfizer Oncology Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and establish the Maximum Tolerated Dose (MTD) and/or Recommended Phase 2 Dose (RP2D) of PF-07934040 as a single agent and in combination with other targeted agents. **Optimization Target:** Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D) **Secondary Objectives:** To evaluate the preliminary anti-tumor activity (including Objective Response Rate [ORR], Duration of Response [DoR], Progression-Free Survival [PFS], and Overall Survival [OS]) and to characterize the pharmacokinetic (PK) profile of PF-07934040.

2.2 Background & Rationale Primary Condition: Advanced Solid Tumors (PDAC, CRC, NSCLC) **Medical Methodology:** Targeted pan-KRAS Inhibition To address the unmet medical need in patients with advanced solid tumors harboring KRAS mutations. KRAS mutations are highly prevalent in cancers such as Pancreatic Ductal Adenocarcinoma (PDAC), Colorectal Cancer (CRC), and Non-Small Cell Lung Cancer (NSCLC). PF-07934040 is an investigational pan-KRAS inhibitor designed to target these mutations in patients who have progressed on standard therapies or as a novel early-line treatment.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Management Pathway:** Dose Escalation and Dose Expansion cohorts **Blinding:** Open-label **Randomization:** N/A (Non-randomized assignment to specific cohorts)

3.2 Planned Enrollment & Duration Target N: 330 participants **Number of Sites:** Multicenter, Global (United States, China, Puerto Rico) **Estimated Study Start:** Mid-2024 **Estimated Primary Completion:** TBD based on enrollment velocity and cohort expansion

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults ≥ 18 years of age. 2. Advanced, unresectable, and/or metastatic or relapsed/refractory solid tumor cancers (specifically PDAC, CRC, and NSCLC). 3. **Diagnosis Threshold:** Documented mutation in the KRAS gene. 4. Presence of at least 1 measurable lesion based on RECIST version 1.1 that has not been previously irradiated. 5. Prior therapy constraints based on cohort (e.g., progressed on platinum-containing chemotherapy and checkpoint inhibitor for NSCLC 2-3L; progressed on fluoropyrimidine/oxaliplatin/irinotecan for CRC 2-3L).

4.2 Exclusion Criteria 1. Active or history of clinically significant gastrointestinal (GI) disease (e.g., ulcerative colitis, Crohn's disease, IBD, GI bleeding). 2. Active or history of pneumonitis/Interstitial Lung Disease (ILD) or pulmonary fibrosis requiring systemic steroid therapy. 3. Diagnosis of immunodeficiency or an active autoimmune disease requiring chronic systemic steroid therapy (>10 mg daily prednisone equivalent). 4. Active bleeding disorder or major surgery/ radiation therapy ≤ 4 weeks prior to enrollment.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: PF-07934040 administered at prescribed dose and frequency in 28-day cycles. Part 1 evaluates monotherapy dose escalation, while Part 2 evaluates monotherapy and combination dose expansions. **Route of Administration:** Oral / Intravenous (dependent on the combination therapies utilized in expansion cohorts, such as cetuximab, pembrolizumab, or chemotherapies). **Duration of Treatment (Total Duration):** Treatment until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications (e.g., antiemetics, pain management) as per institutional guidelines. **Prohibited:** Concurrent treatment with other investigational agents; strong CYP inhibitors/inducers as specified in the protocol; systemic steroids exceeding allowed doses for immunosuppression.

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6. Study Timeline & Visit Schedule

Onsite Visit Cadence: Per 28-day cycle with specific PK/PD and DLT monitoring days **Checklist Review Interval:** Prior to the start of each dosing cycle

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, KRAS mutation confirmation, Baseline Imaging (RECIST v1.1), Safety Labs | | **Baseline** | Day 0 | Final eligibility check, Cohort Assignment | | **Treatment Cycles** | Cycle 1, Day 1 (ongoing every 28 days) | IP Dispensation, PK/PD blood draws, Safety Assessments, DLT monitoring | | **Interim** | Q8W (Every 8 Weeks) | Efficacy Assessment (Tumor Imaging), AE reporting | | **End of Study**| $+ 30$ Days post-last dose | Final Vital Signs, Exit Interview, Safety Follow-up, IP Accountability |

(Note: Cycle duration is standard 28 days unless modified due to dose-limiting toxicities [DLTs])

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of Dose-Limiting Toxicities (DLTs) to determine MTD/RP2D; Objective Response Rate (ORR); and overall safety profile (incidence of Adverse Events). **Measurement Method:** CTCAE v5.0 for safety assessments and RECIST v1.1 criteria via central/local radiological review for ORR.

7.2 Secondary & Exploratory Endpoints 1. Progression-Free Survival (PFS) and Duration of Response (DoR). 2. Overall Survival (OS). 3. Pharmacokinetics (PK) profile of PF-07934040.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring throughout the study utilizing the NCI CTCAE grading system, collected via HCP assessments and laboratory testing. **Serious Adverse Events (SAEs):** Must be reported to the Sponsor and Institutional Review Board (IRB)/Ethics Committee within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An internal Dose Escalation Committee (DEC) / Safety Review Committee (SRC) evaluates safety data between cohorts to approve dose escalation steps.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) / Efficacy Evaluable Set for tumor response endpoints; Safety Set for all participants receiving at least one dose of the investigational product. **Sample Size Justification:** Target enrollment of $N=330$ is based on standard Phase 1 dose-escalation statistical models (e.g., mTPI or 3+3 design) followed by adequately powered expansion cohorts to observe initial efficacy signals. **Handling of Missing Data:** Efficacy endpoints will typically use censoring rules for PFS/OS per standard oncology analysis; missing data will not be imputed for safety analyses.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits by Sponsor/CRO to ensure protocol adherence, correct RECIST assessment, and safety reporting. **Remote Monitoring Frequency:** As needed or permitted per site SOPs. **Quality Audit Mechanism:** Routine clinical site monitoring and central safety data review. **Education Protocol (HCP):** Investigator training on DLT monitoring and protocol-specific AE management. **Education Protocol (Patient):** Informed consent process and toxicity reporting education. **Audit Trail:** Data entry managed through validated electronic Case Report Forms (eCRFs) with comprehensive audit trails and data clarification workflows.

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11. Study Identifiers & Governance

Primary ID: {{PRO00116192}} **Registry Number:** {{NCT06447662}} **Sponsor/Lead Organization:** {{Pfizer}} **Study Title:** {{Phase 1 Study of PF-07934040 in Subjects with Adv Solid Tumors with KRAS Mutation}} **Official Regulatory Title:** {{A PHASE 1 OPEN-LABEL STUDY OF PF-07934040 AS A SINGLE-AGENT AND IN COMBINATION WITH OTHER TARGETED AGENTS IN PARTICIPANTS WITH ADVANCED SOLID TUMORS HARBORING MUTATIONS IN THE KRAS GENE}}

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12. Clinical Framework (Oncology Specific)

Primary Condition: {{Advanced Solid Tumors (PDAC, CRC, NSCLC)}} **Diagnosis Threshold:** {{Documented mutation in the KRAS gene}} **Medical Methodology:** {{Targeted pan-KRAS Inhibition}} **Optimization Target:** {{Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D)}}

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13. Study Procedures & Methodology

Management Pathway: {{Dose Escalation and Dose Expansion cohorts}} **Education Protocol (HCP):** {{Investigator training on DLT monitoring and protocol-specific AE management}} **Education Protocol (Patient):** {{Informed consent process and toxicity reporting education}} **Quality Audit Mechanism:** {{Routine clinical site monitoring and central safety data review}}

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14. Observation & Follow-Up Schedule

Total Duration: {{Treatment until disease progression or unacceptable toxicity}} **Onsite Visit Cadence:** {{Per 28-day cycle with specific PK/PD and DLT monitoring days}} **Remote Monitoring Frequency:** {{As needed or permitted per site SOPs}} **Checklist Review Interval:** {{Prior to the start of each dosing cycle}}

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Insert Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Insert Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]